Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

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INFORMED CONSENT FORM

Septoplasty

Surgery to straighten the central wall of the nose (nasal septum)

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Procedures to be performed at the same time:
- ☐ Turbinate reduction (radiofrequency / submucosal)
- ☐ Endoscopic sinus surgery — FESS (on a separate consent form)
- ☐ None _____

Nasal splints / packs:
- ☐ Silicone splints ☐ Packs ☐ Not needed
- ☐ To be decided during surgery _____

Patient's complaint (side of blockage, snoring, headache, etc.): _____

1 Your condition and the aim of the procedure

The wall of cartilage and thin bone that divides the nasal cavity into right and left sides is called the **septum**. When the septum is bent to one or both sides, this is called a **septal deviation**. It may be present from birth, develop during growth or follow a blow to the nose.

A deviated septum can cause nasal blockage, mouth breathing, snoring, poor sleep, a dry mouth, nosebleeds and, in some people, episodes of sinusitis or headache. Because it is a structural problem, it does not improve with medication. The aim of septoplasty is to straighten the bent parts so that you can **breathe comfortably through your nose**. This operation does not aim to change the outer appearance of the nose.

2 How is the procedure performed?

The operation is usually performed under general anaesthesia in an operating theatre, entirely through the nostrils. There is no visible incision or scar on the face.

1. **Incision:** A small incision is made inside the nostril, in the lining (mucosa) covering the septum.
2. **Lifting the lining:** The lining covering both sides of the septum is gently separated from the cartilage and bone.

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3. **Correcting the deviation:** The bent pieces of cartilage and bone are removed, straightened or repositioned. The supporting parts that hold the outer shape of the nose (the cartilage framework supporting the bridge and tip) are preserved.
4. **Reducing the turbinates (if needed):** If the structures on the side wall of the nose called turbinates are enlarged, they are reduced with radiofrequency or by removing tissue from beneath the lining.
5. **Closure:** The lining is laid back in place and held with dissolvable stitches.
6. **Splints/packs:** To help the septum heal straight and to prevent bleeding, soft silicone splints with a central airway, or packs, are usually placed inside the nose.

If the front and outer part of the septum is also bent, or if the outer shape of the nose contributes to the blockage, the operation may be combined with rhinoplasty (septorhinoplasty), provided this is discussed and consented to separately.

ANAESTHESIA	APPROXIMATE DURATION	HOSPITAL STAY / DISCHARGE
General anaesthesia (in selected cases local anaesthesia + sedation)	About 45–90 minutes	Same day or one night in hospital

3 Expected benefits and likelihood of success

- Easier breathing through the nose and less mouth breathing
- Better sleep quality and less snoring (snoring may also have other causes)
- Fewer recurrent nosebleeds caused by the deviation and, in some patients, fewer episodes of sinusitis
- Better access for sinus or other nasal surgery if needed

Published studies report that about **70–85 out of 100** patients describe a clear improvement in nasal blockage after surgery. If the blockage also has other causes, such as allergy, swelling of the lining, weak nostrils or sinus disease, these may need separate treatment. Rarely, the septum may gradually bend slightly again over time.

4 Other treatment options

- **No surgery / watchful waiting:** If your symptoms are mild, they can simply be monitored.
- **Medical treatment:** Steroid nasal sprays, nasal rinses and allergy medicines can relieve symptoms by reducing swelling of the lining, but they do not correct a structural bend in the bone and cartilage.
- **Nasal strips / nostril dilators:** May give temporary relief.
- **Decongestant sprays:** Suitable only for a few days; long-term use leads to dependence and even worse blockage.

5 What happens if the procedure is not performed?

If the operation is not performed, the deviation will not straighten by itself; symptoms such as nasal blockage, mouth breathing and snoring will continue and may worsen with age. This is usually not dangerous to life; the decision to operate depends on how much your symptoms affect your quality of life.

You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

6 Preparing for the procedure

- Blood tests and, if necessary, a CT scan of the sinuses and other investigations are carried out before surgery, and you will be assessed by the anaesthetist.
- You must not eat or drink anything for **6–8 hours** before the operation (please follow the anaesthetist's instructions).

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- If you develop a cold, flu or a feverish illness in the days before surgery, please let us know; the operation may be postponed.
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.
- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

7 Recovery: what to expect

First 48 hours	Your nose will feel blocked because of the splints, and you can expect a mild blood-stained ooze, headache and a dry throat. Pain is usually mild. Sleeping with your head raised is recommended. The gauze pad under the nose may need changing a few times.
Days 3–7	Silicone splints or packs are usually removed at a follow-up visit between **days 3 and 7**; this causes brief discomfort. Nasal rinsing is started. Most people return to desk work within 5–7 days.
Weeks 2–4	Crusting and dryness inside the nose may occur and settle with regular rinsing. Breathing gradually improves. Strenuous exercise and contact sports should be postponed for about **3–4 weeks**.
Months 1–3	The nasal lining heals completely. The final breathing result is usually assessed during this period.

8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

Expected, temporary effects (not complications — a normal part of healing)

- Nasal blockage and mouth breathing until the splints are removed
- Mild blood-stained ooze in the first few days
- Temporary numbness of the upper front teeth and upper lip
- Crusting and dryness inside the nose (in the first few weeks)
- Temporary reduction in sense of smell

Common conditions (most are temporary or resolve with treatment)

- **Prolonged crusting and dryness** — May last several weeks, especially if turbinate reduction was performed; settles with rinsing and moisturising ointment.
- **Prolonged numbness of the upper teeth and lip** — Usually resolves within a few weeks; rarely it may last a few months.

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- **Blockage not fully resolving** — In about **15–30 out of 100** patients, breathing may not improve as much as expected. This is often due to additional factors such as allergy, swelling of the lining or weak nostrils; medical treatment or further surgery may be needed.

Uncommon conditions

- **Nosebleed** — In about **1–3 out of 100** patients, bleeding requiring packing may occur. Very rarely, the bleeding may need to be stopped in the operating theatre.
- **Hole in the septum (perforation)** — In about **1–3 out of 100** patients, a hole may form in the septum. It often causes no symptoms; if it causes whistling, crusting or bleeding, rinsing, a septal button or a repair operation may be needed.
- **Adhesions inside the nose (synechiae)** — In about **1–5 out of 100** patients, adhesions may form between the septum and the side wall; if they cause blockage they are separated with a simple procedure at follow-up.
- **Collection of blood or pus in the septum (haematoma / abscess)** — Occurs in **fewer than 1 in 100** patients. It shows as increasing blockage, pain and fever in the days after surgery. It must be **drained urgently** to avoid damage to the nasal cartilage; if untreated, loss of cartilage and collapse of the nasal bridge may occur.
- **Infection** — Rare; it usually settles with antibiotics.
- **Recurrence of the deviation** — Because cartilage has a “memory”, the septum may bend slightly again over time; if this causes symptoms, a second operation may be needed.

Rare or very rare, but potentially serious conditions

- **Collapse of the nasal bridge (saddle nose)** — Occurs in **fewer than 1 in 100** patients. Weakening of the supporting cartilage of the septum, or loss of cartilage after a haematoma/abscess, can cause the bridge of the nose to collapse; this changes the outer appearance of the nose and requires further surgery with a cartilage graft to correct.
- **Drooping or change in shape of the nasal tip** — Very rare; may occur if supporting structures are affected.
- **Permanent reduction or loss of sense of smell** — Very rare.
- **Cerebrospinal fluid (CSF) leak and meningitis** — Because the upper part of the septum is next to the skull base, this is an extremely rare complication occurring in **far fewer than 1 in 10,000** cases. If it occurs, hospital admission, further treatment and a repair operation may be needed.
- **Empty nose syndrome** — If the turbinates are reduced excessively, a feeling of blockage and dryness may develop even though the nose is open. This is very rare; for this reason the turbinates are reduced conservatively. If it develops, it may be long-lasting or permanent and is difficult to treat.
- **Toxic shock syndrome** — A very rare but serious illness linked to the use of nasal packs/splints (sudden high fever, rash, low blood pressure) that needs urgent treatment. It requires hospital treatment, sometimes in intensive care, and can be life-threatening.
- **Visual disturbance, bleeding around the eye** — Extremely rare with septoplasty; the risk is slightly higher when combined with sinus surgery such as FESS and is explained on a separate consent form.

General risks that apply to any surgical / medical procedure

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.
- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.

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- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

9 When to contact us immediately

- **Nosebleed that does not stop or is very heavy** despite a few minutes of pressure and ice, or blood running continuously into the mouth
- **Increasing nasal blockage, severe pain and swelling of the nose** in the days after surgery (may be a sign of a septal haematoma/abscess)
- **Fever above 38.5 °C** or foul-smelling discharge
- **Sudden high fever, skin rash, dizziness or weakness** while packs/splints are in place
- **Clear, watery discharge** from the nose, especially if it increases when you lean forward; severe headache or neck stiffness
- **Blurred or double vision**, or swelling around the eye
- Shortness of breath or chest pain

If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).

10 Medication and everyday-life advice

- Do not blow your nose for the first 2 weeks; keep your mouth open when sneezing.
- Rinse your nose as shown to you and as often as recommended.
- For the first 2 weeks avoid hot baths, saunas, Turkish baths and heavy lifting.
- Protect your nose from knocks for at least 4 weeks.
- If you have travelled from abroad, flying is usually safe once the splints have been removed.
- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.
- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.
- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

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11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

12 Photographs, image records and your personal data

Photographs/videos may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

- ☐ I agree that my images are used **only for medical records and treatment follow-up**.
- ☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).
- ☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.
- ☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

13 Information and notes specific to you (completed by the doctor)

Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write “none”.

14 Patient declaration

- ☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.
- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.

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- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.
- ☐ I understand that the aim of this operation is to improve breathing, not to change the outer appearance of the nose, and that further treatment may be needed if the blockage has other causes.

- ☐ **I GIVE MY CONSENT to the procedure described above.**
- ☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."

15 Signatures

	FULL NAME	SIGNATURE	DATE / TIME
Patient			
Legal representative (parent / guardian) — relationship:			
Doctor providing information and performing the procedure			
Interpreter (if needed) — language:			
Witness (if needed)			

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

- ☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.
- ☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

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This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.